

Suicide Risk Screening and Prevention

Universal Screening, Safety Planning, and Ligature Risk Management

Regulation: 42 CFR §482.13(e) | Joint Commission NPSG 15.01.01

POLICY OVERVIEW

Inpatient suicide is a sentinel event and a serious patient safety concern. Hospitals must implement universal suicide risk screening, provide safe environments through ligature risk reduction, and establish individualized safety plans for at-risk patients regardless of the reason for admission.

KEY REQUIREMENTS

1 Universal Screening

All patients presenting to the emergency department and all inpatients admitted to behavioral health units must be screened for suicide risk using a validated tool (Columbia Suicide Severity Rating Scale [C-SSRS] or ASQ). Medical/surgical inpatients should be screened if risk factors are identified.

2 Risk Stratification and Interventions

Screening results must be used to stratify risk (low, moderate, high) and determine appropriate interventions. High-risk patients require: 1:1 continuous observation, removal of potentially harmful items, psychiatric consultation, and immediate safety planning.

3 Environmental Safety / Ligature Risk

Patient care areas where patients at risk are treated must be assessed for and mitigated against ligature risks (points where a cord, rope, or other material can be looped to create a hanging risk). Mitigation measures must be documented and regularly evaluated.

4 Safety Planning

Individualized safety plans must be developed collaboratively with the patient, documented in the medical record, and updated as clinical status changes. Safety plans must address: warning signs, coping strategies, reasons for living, support contacts, and means restriction.

5 Transition Care and Means Restriction Counseling

At discharge, patients identified as at risk must receive: crisis resources (988 Lifeline), follow-up appointment within 7 days (Joint Commission standard), and means restriction counseling addressing access to lethal means (firearms, medications).

COMPLIANCE NOTES

Inpatient suicides are reportable sentinel events to The Joint Commission. Hospitals must report to state authorities as required. Following any inpatient suicide, a root cause analysis must be completed and action plan implemented.

COMMON SURVEY CITATIONS

Required under Joint Commission NPSG 15.01.01 – Suicide risk identification, and §482.13(e) – Safe environment standards.